National Cancer Control Programme (NCCP) 00689 · clinician draft

Atezolizumab + carboplatin + etoposide

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Induction Day 1 · carboplatin AUC 5

Active medicines: atezolizumab + carboplatin + etoposide

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before treatment Day 1, then daily Days 2–3
Ondansetron	16 mg orally	Day 1 Individualise for QT risk, interactions and patient factors.
Dexamethasone	12 mg orally Day 1; 8 mg orally daily Days 2–4	Carboplatin AUC≥4 dedicated NCIS V8 row Reconcile steroid indications and aprepitant interactions; one clear prescription.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS carboplatin row does not define an end day; confirm duration, renal suitability and contraindications.

Additional phase-specific support

- Dedicated carboplatin AUC≥4 row has no routine olanzapine; individualise if clinically indicated.
- Reconcile continued Day 1 prophylaxis with Days 2–3 etoposide cover and the two days after the last chemotherapy day. Do not duplicate antiemetics.
- No routine atezolizumab premedication unless previous reaction; after Grade 1–2 reaction consider antipyretic/antihistamine and close monitoring.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00689 v4 · p10 · NCCP NCIS Medical Oncology antiemetics V8

Sources checked 2026-10-29 · SACTCheck v0.84.26 · Regimen-level drafts; prescriber and oncology-pharmacy verification required